



GHK-CU RESEARCH HUB · RESEARCH USE ONLY

FIRMER SKIN. STRONGER HAIR.

FOR MEN & WOMEN. Get firmer skin, stronger hair, fewer wrinkles, and faster wound healing.

[Get GHK-Cu 50 mg at BioLinx](#)

[Read the guide first](#)

Also shown to help with: aging skin · acne · alopecia · stretch marks · ehlers-danlos
Once daily · time of day doesn't matter · Difficulty: beginner

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RESEARCH SHOWED THAT...

55% less wrinkle

volume
vs. control after 8 weeks.

Int J Mol Sci, 2018

Wrinkle volume dropped 55%
compared to the control group
after 8 weeks of GHK.

Pickart & Margolina, Int

J Mol Sci, 2018



STEP 1 OF 4 · WHAT TO BUY

SHOPPING LIST

Easy-to-follow shopping list to safely practice GHK-Cu. Match your commitment level — don't skip anything.



PEPTIDE · ONE ORDER

GHK-CU 50 MG

Get firmer skin, stronger hair, fewer wrinkles, and faster wound healing. Common starting dose: **1 mg**, once daily, time of day doesn't matter.

Get GHK-Cu 50 mg at BioLinX

biolinxlabs.com/products/ghk-cu-50-mg

HOW MANY VIALS TO BUY · BEST VALUE BY GOAL

GOAL	VIAL	QTY	WHERE
1 MONTH	50 mg vial	1×	<u>See it</u> biolinxlabs.com/products/ghk-cu-50-mg
FULL CYCLE RECOMMENDED	50 mg vial	2×	<u>See it</u> biolinxlabs.com/products/ghk-cu-50-mg

MATERIALS ≈ \$41 ON TOP — FULL LIST ON THE NEXT PAGE



PRODUCTS ARRIVED?

QUICK SUMMARY OF YOUR LAB ITEMS:

TWO VIALS IN THE BOX • GHK-CU 50 MG + BACTERIOSTATIC WATER 10 ML

VIAL 1	GHK-Cu 50 mg. Arrives as a powder. Don't touch it until you've read page 4.
VIAL 2	Bacteriostatic water — 10 mL. This is what turns the powder into a liquid. Sterility and pH matter — don't substitute.

GROCERY LIST • ≈ \$41

ITEM	SPEC	QTY	WHERE
INSULIN SYRINGES BUY	1cc U100, 30G, 5/16"	50–100 ct	Any pharmacy
ALCOHOL PREP PADS BUY	Any brand, 70% isopropyl	50–100 ct	Any pharmacy
SHARPS CONTAINER BUY	1 quart, for needle disposal	1×	Any pharmacy
BAC WATER BUY	Bacteriostatic water, 10 mL — sterility and pH matter, don't substitute	1 vial	<u>See it</u> biolinxlabs.com/products/bacteriostatic-water-10-ml

Any brand works for syringes, pads and sharps — but not BAC water. Use 2 fresh syringes and 2 fresh pads every session: one to mix, one to inject.

HOW TO STORE IT

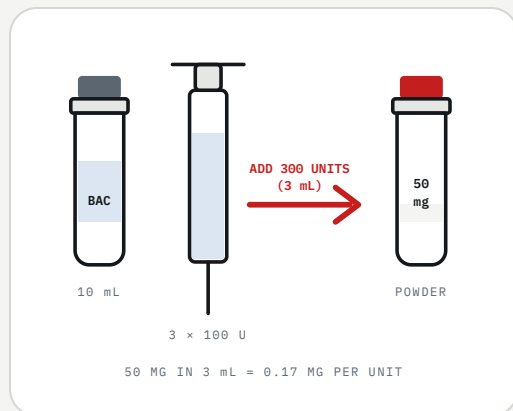
POWDER	Freezer — lasts years
RECONSTITUTED	Fridge — about 28 days. Write the mix date on the vial.
WATER	Room temperature — check the expiration date



STEP 2 OF 4 · RECONSTITUTION – FROM POWDER TO LIQUID

LET'S “FORM” YOUR PEPTIDE!

“Form” means “reconstitute”. Your product arrives as a powder — you need to liquify it before “you begin your research.” 100 units = one full syringe. **50 mg vial → add 300 units (3 mL)**. Every dose in this guide assumes exactly that ratio, so measure it — don't eyeball it.



YOUR SIMPLIFIED UNITS

DOSE	FILL TO
------	---------

1 mg	6 units
------	---------

2 mg	12 units
------	----------

2.5 mg	15 units
--------	----------

**Change the vial, water or dose →
interactive calculator**

1

Remove the plastic caps & wipe both rubber vial tops

Fresh alcohol pad on the GHK-Cu vial and the bac water vial.

2

Draw 300 units of bac water

That's 3 full pulls on a 100-unit insulin syringe (3 mL total).

3

Inject it slowly down the side of the 50 mg vial

Let it run down the glass. Don't blast the powder.

4

Swirl gently. Never shake. Place it in the fridge!

Slow and gentle wins this one — shaking wastes peptide. Discard that syringe into the sharps container. Let it rest until completely clear (36–46 °F); use within ~28 days; never freeze it.



STEP 3 OF 4 · INJECT

WHERE TO INJECT

Time to inject · clean, pinch, poke, done. Once daily, time of day doesn't matter.

1

Take GHK-Cu out of the refrigerator and extract your dose

Your first dose is **1 mg = 6 units** on a 100-unit syringe. Wipe the vial top with a fresh alcohol pad, let it air-dry a few seconds, then draw your units.

2

Pinch a fold of skin on the stomach + use an alcohol wipe

Two inches away from the belly button. Wipe with a fresh alcohol pad, let it air-dry, and don't touch that spot again before injecting. Rotate sites.

3

Needle in at the base of the fold

Push the plunger slowly and steadily. Hold for a few seconds, then withdraw.

4

That's it — you just did your first shot!

Drop the needle straight into the sharps bin. Vial goes back in the fridge.



STOMACH · PINCH A FOLD · NEEDLE AT THE BASE

WATCH FIRST

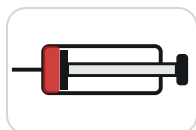
Mixing and injecting from start to finish, on video: professorpeptides.com. Use 2 fresh syringes and 2 fresh pads every session — one to mix, one to inject.



STEP 4 OF 4 · DOSING & TIMING SCHEDULE

GHK-CU SCHEDULE (PROTOCOL)

“Schedule” means “protocol”. Everything below assumes the 50 mg vial was reconstituted with exactly 300 units (3 mL), read off a standard 100-unit insulin syringe.



FIRST DOSE

How much, how often & when?

WHEN

Time of day doesn't matter. Once daily.

HOW MUCH

1 mg to start.

Dose runs in on/off blocks with breaks between — this is **cycling**, not titration. Full schedule on page 9.

UNITS

Extract **6 units** on a 100-unit insulin syringe.

6 units = 1 mg. Exact variables: 100-unit syringe · 50 mg vial reconstituted with 300 units · 0.17 mg per unit.

HOW OFTEN

Once daily. Hold each step for its full block before moving up.

CYCLE

1 month on, 2 months off.

Don't rush it — start at the lowest amount, hold each step for its full block, and only step up if the current dose is comfortable. Most people never need the top doses.



STEP 4 OF 4 · DOCUMENT

WHAT TO EXPECT + SIDE EFFECTS

Mostly mild, mostly early, mostly right after a dose increase. Know the three tiers before the first shot, not after.

NORMAL / EXPECTED

First few days

- Redness or mild swelling at the injection site (10–20% of users)
- Mild headache or light-headedness for 30 minutes to a few hours
- A brief metallic taste right after the shot — a copper effect

COMMON, WORTH NOTING

Manage it, don't push the dose

- Blood-pressure dips that cause dizziness — stand up slowly, hydrate
- Mild nausea at higher doses
- Bruising at the site — rotate spots

STOP & CALL A DOCTOR

Don't push through it

- Dizziness that doesn't pass, or fainting
- Signs of an allergic reaction — swelling, rash, difficulty breathing
- Any sign of copper overload — nausea with stomach pain, or yellowing skin/eyes

NOTE: for this one it is **HIGHLY RECOMMENDED** to start at the lowest amount first and work up over time, as needed. Not for anyone with Wilson's disease or other copper-handling disorders, or who is pregnant or breastfeeding — talk to a doctor before use.



FIRST-TIME TL;DR

RECAP: CHECKLIST

Everything from pages 2–7, in the order it happens.

BEFORE

- ☐ Vial reconstituted with exactly 300 units of bac water, resting in the fridge, mix date written on it
- ☐ Supplies laid out: new syringe, alcohol pad, sharps container
- ☐ Dose set at **1 mg = 6 units** — not higher, no matter what a forum says

DURING

- ☐ Inject once daily, time of day doesn't matter — stomach, rotate the spot
- ☐ Note the date and dose somewhere — a phone note is fine

AFTER

- ☐ Expect possible redness, mild irritation or a headache — it settles
- ☐ Mild nausea can happen after a dose increase — sit tight, it typically passes
- ☐ If anything feels more than mild — dizziness, real nausea, allergic reaction — stop and get medical advice

NEXT BLOCK

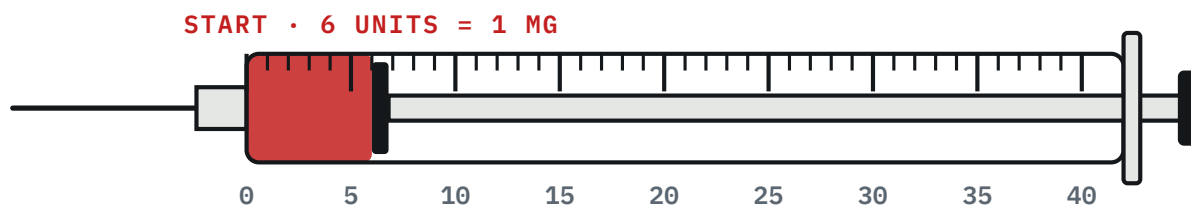
- ☐ Hold each step for its full block before stepping up — never skip a step (1 month on, 2 months off)
 - ☐ Only step up if the current dose is tolerated; if side effects are rough, stay another block
 - ☐ Fresh syringe, fresh pad, needle in the sharps bin — every single time
-

UNIT CHEAT-SHEET · 100-UNIT SYRINGE · 50 MG VIAL RECONSTITUTED WITH 300 UNITS

HOW TO CYCLE (RAMP-UP & CYCLE SCHEDULE)

Assumes the 50 mg vial was mixed with exactly 300 units, so **1 unit = 0.17 mg**.

Units shown are per shot.



WHEN	DOSE	SYRINGE · PER SHOT	NOTES
Month 1	1 mg/day · 7 mg/week	6 units	START HERE
Months 2-3	Off	—	REST
Month 4	2 mg/day · 14 mg/week	12 units	MAX DOSE
Months 5-6	Off	—	REST

Rule of thumb: don't move up a step until you know how the current dose feels. Only step up if the previous block was tolerated. Most people never need to go past the middle of the ramp.



THE GHK-CU SOURCING GUIDE

PROFESSOR PEPS: SOURCING GUIDE (APPENDIX)

How to verify any supplier in 90 seconds. A legitimate source clears all three checks below in under two minutes — no back-and-forth required.



1

Ask for the COA

A batch-specific Certificate of Analysis — not a generic PDF reused across batches — showing purity $\geq 98\%$ via HPLC/MS.

2

Match the lot number on the vial

It must match the lot number on the COA. If they don't match, walk away.

3

Check the track record

Searchable reviews or forum mentions older than roughly six months. Brand-new storefronts with no history are the highest-risk category.

DON'T WORRY — WE'VE ALREADY DONE THE VETTING

BioLinx Labs clears the list: third-party HPLC testing, a 99%+ purity guarantee and a COA included with every order. biolinxlabs.com/best-sellers

biolinxlabs.com/best-sellers

Educational and research purposes only — not medical advice. Professor Peptides does not sell peptides. Products referenced are sold for laboratory research use only and are not for human consumption. Want to learn more? Join the Professor Peptides community — 30 days free.

STAY CURIOUS, PROFESSOR PEPTIDES

GHK-Cu Research Hub · companion to the email series